

University of Maryland - CS2 Trend

Maryland Next Gen NCLEX Test Bank Project September 1, 2022; Author: Elizabeth Mackessy-Lloyd, DNP, RN, CNE, Hood University. Stand-alone trend for the Tuberculosis case (University of Maryland - CS2): changes between the first visit and the 4-week follow-up.

Taken from the Maryland Next Gen NCLEX Test Bank Project, University of Maryland School of Nursing: "Tuberculosis" medical-surgical faculty case study, stand-alone trend (Elizabeth Mackessy-Lloyd, DNP, RN, CNE, Hood University, September 1, 2022). Available at <https://www.nursing.umaryland.edu/mnwc/initiatives/nextgen-nclex/nextgen-nclex-library/> (Faculty Case Studies). Laboratory values shown in SI units.

Draft review. Each screen scores full marks with its answer key and keeps every chart tab and entry on later screens. Added hidden from students (draft) until reviewed. Each screen is shown as students see it, then answered with the key; the rationale follows.

Chart tabs (last screen)

NOT COMPLETE

A 48-year-old female client with tuberculosis is seen in the clinic at a 4-week follow-up appointment.

Nurses' Notes

Laboratory and Diagnostic Results

Orders

Laboratory Test and Reference Range	April 25
White blood cell (WBC) count 5.0–10.0 × 10 ⁹ /L	12.0 × 10 ⁹ /L
Platelet count 150–400 × 10 ⁹ /L	358 × 10 ⁹ /L
Hemoglobin (Hgb) 115–155 g/L	108 g/L
Hematocrit (Hct) 0.36–0.48 L/L	0.35 L/L
Hemoglobin A1c (HbA1c) < 5.7%	6.0%
Cholesterol, total < 5.2 mmol/L	5.7 mmol/L
Aspartate aminotransferase (AST) 9–32 U/L	30 U/L
Alanine aminotransferase (ALT) 19–25 U/L	21 U/L
Sputum culture Negative	Pending

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April 25: Rifapentine 1200 mg PO daily
Moxifloxacin 400 mg PO daily
Isoniazid 300 mg PO daily
Pyrazinamide 2000 mg PO daily

Practice NCLEX RN Test

Question 1 of 1

Calculator

Question Navigator

NOT COMPLETE

A 48-year-old female client with tuberculosis is seen in the clinic at a 4-week follow-up appointment.

Nurses' Notes

Laboratory and Diagnostic Results

Orders

April 25: Reports recent extended travel to Asia with a tour group. History of coughing, mild fatigue, and loss of appetite since return, 3 weeks ago. Complains of pain in chest of 4/10 on coughing. Reports taking an over-the-counter cough medication with limited results. VS: BP 120/76 sitting, 118/72 standing, HR 78 beats per minute and regular, T 100°F (37.8°C) orally, RR 20, pulse oximetry reading 95% on room air. Lung sounds are diminished bilaterally with mild crackles noted in the bases. Weight 210 lb (95 kg), BMI 30. Labs and chest X-ray obtained.

May 25: Client returns for a follow-up appointment 4 weeks after being diagnosed with a tuberculosis infection. Reports missing several doses of medication. Continues to have a productive cough and is tired most days. Rates pain with the cough at 3/10, but now has new abdominal pain.

VS: T 98.8°F (37.1°C), P 80, RR 22, BP 144/88, pulse oximetry reading 95% on room air. Repeat WBC $9.0 \times 10^9/L$.

➤ For each finding, click to specify if the finding indicates the client's status has improved, declined, or is unchanged.

Finding	Improved	Declined	Unchanged
Missing several doses of medication	☐	☐	☐
Pulse oximetry reading	☐	☐	☐
Productive cough	☐	☐	☐
Blood pressure	☐	☐	☐
Temperature	☐	☐	☐
Fatigue	☐	☐	☐
WBC count	☐	☐	☐
Pain characteristics	☐	☐	☐

Note: Each row must have only 1 response option selected.

Submit

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Quit

Previous

Finish attempt →

Practice NCLEX RN Test

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Productive cough	☐	☐	☒ ✓
Blood pressure	☐	☒ ✓	☐
Temperature	☒ ✓	☐	☐
Fatigue	☐	☐	☒ ✓
WBC count	☒ ✓	☐	☐
Pain characteristics	☐	☒ ✓	☐

Note: Each row must have only 1 response option selected.

✓ your correct answer ✗ your incorrect answer ✓ correct answer you did not choose

Submit

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Finish attempt →

Rationale: The elevated blood pressure and the missed doses of medication are cause for further assessment and client education. The new abdominal pain could be a sign of a medication side effect or liver involvement. The temperature is now normal, and the WBC count is within normal limits. The pulse oximetry reading (95% on room air), the productive cough, and the fatigue are unchanged from the initial visit.